TITLE AND ENACTING CLAUSE

IN THE SENATE OF THE UNITED STATES A BILL To strengthen critical access hospitals in medically underserved rural communities and cap out-of-pocket prescription drug costs for senior citizens. Be it enacted by the Senate and House of Representatives of the United States of America in Congress assembled,

SECTION 1. SHORT TITLE.

This Act may be cited as the 'Rural Healthcare Access and Senior Prescription Affordability Act of 2026'.

SECTION 2. SENIOR PRESCRIPTION MEDICATION CO-PAY CAPS.

(a) Monthly Cost Ceiling.·Under Medicare Part D, a participating beneficiary aged 65 or older shall not be required to pay a co-payment exceeding $35 for a 30-day supply of insulin, cardiovascular maintenance therapeutics, or inhalers. (b) Elimination of Deductibles.·The standard Medicare deductible shall not apply to qualifying tier-1 life-saving chronic medications. (c) Penalty for Non-Compliance.·Pharmaceutical manufacturers that artificially elevate wholesale acquisition costs above the regional inflation adjustment rate shall be subject to a 95 percent excise rebate penalty.

SECTION 3. RURAL CRITICAL ACCESS HOSPITAL CAPITAL GRANTS.

(a) Capital Assistance Facility.·The Administrator of the Health Resources and Services Administration (HRSA) shall award non-matching grants of up to $2,500,000 per facility to non-profit hospitals located in counties with fewer than 35,000 residents. (b) Allowable Expenditures.·Funds may be utilized for emergency room modernization, digital diagnostic telemetry, and nurse recruitment signing bonuses not to exceed $25,000 per registered nurse. (c) Mobile Telehealth Outreach.·Grants up to $350,000 shall be made available to local municipal public libraries to deploy private, HIPAA-compliant virtual telemedicine consultation kiosks.